5.4 Increased Intracranial Pressure

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Says his headache is getting worse **Objective** • GCS falling • Projectile vomiting • BP 180/70 mmHg, HR 52/min (pulse pressure widening)	**ACTUAL** **Decreased intracranial adaptive capacity** related to increased ICP as evidenced by decreasing GCS, vomiting and Cushing response.	ICP and cerebral perfusion pressure stay within the prescribed limits.	• Report signs of rising ICP at once. • Head of the bed at 30 degrees, head in the midline. • Avoid anything that pushes ICP up: suctioning longer than needed, coughing, straining. • Give osmotic therapy as prescribed. • Keep temperature and blood glucose normal.	Neurological status stable: **goal met**.
Subjective • *No specific complaint* **Objective** • Breathing irregular • SpO_2 90%	**ACTUAL** **Ineffective breathing pattern** related to pressure on respiratory center as evidenced by irregular respirations and low SpO_2.	SpO_2 94% or more and breathing regular.	• Keep the airway open and give oxygen as prescribed. • Continuous monitoring of RR, breathing pattern and SpO_2. • Do not let CO_2 build up, because hypercapnia raises ICP. • If ordered, prepare for intubation.	SpO_2 on target: **goal met**.
Subjective • *No specific complaint* **Objective** • Not fully conscious • Vomiting	**RISK** **Risk for aspiration** as evidenced by risk factors: decreased consciousness and vomiting.	Airway stays clear, with no aspiration.	• Turn him on his side unless there is a reason not to. • Have suction ready at the bedside. • Nothing by mouth until his swallow has been checked.	No aspiration: **goal met**.